



Primary Health Care (PHC) &SDGs

Berhan T.(PhD fellow at
MLU,Germany , SLL Manager

OUTLINE

- **Definition**
- **Historical development**
- **Approachs**
- **Principles**
- **The PHC philosophy**
- **Strategy**
- **The level of care**
- **The components/elements of PHC**
- **PHC in Ethiopia**

Primary Health Care (PHC)

Definition

- PHC is essential health care based on practical, scientifically sound, and socially accepted methods and technology made universally accessible to individual and families in the community through their full participation and at a cost that the community and country can afford to maintain at every stage of their development in the spirit of self reliance and self determination.

Main terms in the definition

1. Essential – basic and indispensable
2. Scientifically sound – scientifically explainable and acceptable
3. Socially acceptable methods and technology – acceptable by the community and consider the local value, culture and belief
4. Universally accessible – to bring health care as close as possible to where people live and work
5. Community involvement – individual and families assume greater responsibility for their own health.
6. Self reliance and self determination- Being independent understanding your own needs and trying to minimize problems.

Historical development

- WHO was established in 1948.

Major objective was the attainment by all people of the highest possible level of health

- Health care approaches implemented in different countries between 1948 and 1978 did not enable WHO to meet its objective
- In the 1950s
 - Vertical health service strategy was implemented (control of malaria, TB and venereal diseases) - *Was expensive and unsuccessful.*
 - Strategy of health service – construction of health centers and health stations.

- ◉ **In the early 1970s**

- ◉ Integration of the vertical control programs
- ◉ **Disease oriented** based on high cost, requiring advanced technology to solve health problems
- ◉ Health status of hundreds of millions of people in the world is unacceptable
- ◉ Tremendous strides in medicine and technology didn't solve health problems of most people
- ◉ The organized limited health institutions failed to meet the demand of most people

PHC...cont'd

- The health services often created in isolation neglecting other sectors, which are relevant to health.
- Health institutions stressed **curative services**.
- The community has rarely been participated.
- In the late 1970s
 - 1977 - “Health for all by the year 2000.”
 - 1978 – Strategy to meet the above goal was defined
 - PHC in Alma-Ata USSR.

Comprehensive vs selective PHC

Selective PHC approach

- ❖ ***States that*** health services should be directed toward controlling those diseases producing the largest number of deaths and disability.

- ❖ Why selectively focuses on these problems?
- ✓ Limited resources, both human and material
- ❖ It sees health as the **absence of disease**
Generally, it is diseases oriented.

what is the difference b/n this approach and the previously rejected vertical and basic health care strategies?

Comprehensive PHC approach

- ❖ It looks at health *holistically*
- ❖ The approach is **development oriented** and sees health as part of the development
- ❖ broad philosophy or approach to health care and a strategy for organising **health care systems** and **society to promote health**.
- ❖ It promotes **equity**

It explicitly advocates for:

- ❖ universal accessibility and coverage on the basis of need;
- ❖ comprehensive care with the emphasis **on disease prevention and health promotion;**
- ❖ community and individual involvement and self-reliance;
- ❖ intersectoral action to address the social and environmental determinants of health

- ❖ appropriate technology and cost-effectiveness in relation to the available resources
- ❖ social justice
- stressed the the importance of addressing the underlying **social, economic and political causes of poor health**

Characteristic	Selective	Comprehensive
Main aim	Reduction of specific disease	Improvement in overall health of the community and individuals
Strategies	Focus on curative care, with some attention to prevention and promotion	Comprehensive strategy with curative, rehabilitative, preventive and health promotion that seeks to remove root causes of health
Planning and strategy development	External, often 'global', programmes with little tailoring to local	Local and reflecting community priorities professional 'on tap not on top'

Participation

Limited engagement,
based
on terms of outside
experts
and tending to be
sporadic

Engaged participation
that
starts with community
strengths and the
community's
assessment of
health issues, is
ongoing and
aims for community
control

Engagement with
politics

Professional and
claims to be
apolitical

Acknowledges that
PHC is
inevitably political and
engages with local
political
structures

Forms of evidence

**Limited to assessment
of
disease prevention
strategy
based on traditional
epidemiological
methods,
usually conducted out
of
context and
extrapolated to
situation**

**Complex and varied
research
methods including
epidemiology and
qualitative
and participatory
methods**

PHC Principles

1. Inter-sectoral collaboration
2. Community participation
3. Appropriate technology
4. Equity
5. Focus on prevention of disease and health promotion
6. Decentralization

1. **Inter-sectoral collaboration**

- ◉ Joint concern and responsibility of sectors in identifying problems, programs and undertaking tasks.
- ◉ Health has several dimensions that can be affected by other sectors.

2. **Community involvement**

- The communities should be actively involved:
 - In the assessment of the situation
 - Problem identification
 - Priority setting and making decisions
 - Sharing responsibility in the planning, implementing, monitoring and evaluation.

Advantages of community participation

- ◉ Extend service/better coverage
- ◉ Programs are affordable and acceptable
- ◉ Promote self reliance and confidence
- ◉ Create sense of responsibility
- ◉ Consider real needs and demands
- ◉ Promote local community initiatives and technology
- ◉ Reduce dependency on technical personnel
- ◉ Build the community's capacity to deal with problems

Skills for enhancing community participation

- ◉ Belief in community's potential
- ◉ Skills in participatory approach (look, listen and learn)
- ◉ Ability to motivate
- ◉ Awareness creation among leaders
- ◉ Understanding community culture
- ◉ Identify or create a mechanism for community participation.

3. Appropriate technology

➤ Criteria for appropriateness:

- Effective
- Culturally acceptable and valuable
- Affordable, i.e., cost effective
- Locally sustainable
- Environmentally accountable
- Measurable
- Politically responsible

4. Equity

- Universal coverage of the population with care provided according to need.
- Possible definition of equity includes:
 - Equal health
 - Equal access to health care
 - Equal utilization of health care

.

5. Focus on prevention and promotive health services

- Balanced allocation of resources to preventive and curative care

6. Decentralization

- Taking decision making closer to the communities served and to the field providers of services.
- Enhance the ability to tap new sources for financing health care.

The PHC Philosophy

A. Equity and justice

- ◉ Equitable distribution of services, resources and health care.
- ◉ Health service should be accessible and affordable.

B. Individual and community self-reliance

- ◉ Personal responsibility for their own and their family's health.

C. Inter-relationship of health and development

- ◉ Health makes a fundamental contribution to a country's economy.

The PHC Strategy

1. Changes in the health care system

- ◉ Total coverage with essential health care
- ◉ Integrated systems
- ◉ Involvement of communities
- ◉ Use and control of resources
- ◉ Redistribution of existing resources
- ◉ Reorientation of health human resources
- ◉ Legislative changes
- ◉ Design, planning and management of health system

2. Individual and collective responsibility for health

- ◉ Political issue – decentralization of decision-making.
- ◉ Self realization – personal responsibility for their own and their family's health.
- ◉ For both aspects it is important to have informed and motivated public.

PHC – The level of care

Primary level

- ◉ Primary care provided by PHCU and district hospital
- ◉ Levels of prevention – primary, secondary and tertiary.

Intermediate level

- ◉ Secondary care provided by zonal and regional hospitals.
- ◉ Levels of prevention - primary, secondary and tertiary.

Central level

- ◉ Tertiary care provided by central referral and teaching hospital.
- ◉ Levels of prevention – Tertiary prevention.

The components/elements of PHC

1. Health education
2. Provision of essential drugs
3. Immunization
4. MCH/FP
5. Treatment of common diseases and injuries
6. Adequate supply of safe water and basic sanitation
7. Communicable disease control
8. Food supply and proper nutrition

- Additional elements incorporated after Alma-Ata

- 1.Oral health

- 2.Mental health

- 3.The use of traditional medicine

- 4.Occupational health

- 5.HIV/AIDS

- 6.ARI

PHC in Ethiopia

- Formally began in 1980s and includes the following:
 1. Education on the prevailing health problems and methods of preventing and controlling them
 2. Locally endemic diseases prevention and control
 3. EPI
 4. MCH/FP
 5. Essential drugs provision
 6. Nutrition promotion of food supply
 7. Treatment of common diseases and injuries
 8. Sanitation and safe water supply

Major problems in the implementation of PHC in Ethiopia

- ◉ Absence of infrastructure at the district level
- ◉ Difficulty in achieving inter sectoral collaboration
- ◉ Inadequate health service coverage and mal-distribution for available health services
- ◉ Inadequate resource allocation
- ◉ Absence of clear guidelines or directives on implementation
- ◉ Presence of culturally dictated HTP or unscientific beliefs
- ◉ Absence of sound legal rules to support env't health activities
- ◉ Weak community involvement in health

- The 2030 Agenda for Sustainable Development, adopted by all United Nations members in 2015, created 17 world Sustainable Development Goals (SDGs).
- The aim of these global goals is "peace and prosperity for people and the planet" – while tackling climate change and working to preserve oceans and forests.

1 NO
POVERTY



2 ZERO
HUNGER



3 GOOD HEALTH
AND WELL-BEING



4 QUALITY
EDUCATION



5 GENDER
EQUALITY



6 CLEAN WATER
AND SANITATION



7 AFFORDABLE AND
CLEAN ENERGY



8 DECENT WORK AND
ECONOMIC GROWTH



9 INDUSTRY, INNOVATION
AND INFRASTRUCTURE



10 REDUCED
INEQUALITIES



11 SUSTAINABLE CITIES
AND COMMUNITIES



THE GLOBAL GOALS

For Sustainable Development

12 RESPONSIBLE
CONSUMPTION
AND PRODUCTION



13 CLIMATE
ACTION



14 LIFE BELOW
WATER



15 LIFE
ON LAND



16 PEACE AND JUSTICE
STRONG INSTITUTIONS



17 PARTNERSHIPS
FOR THE GOALS



- Thank u